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AU PAIR & ELDERLY CARE GIVING

A Comprehensive Course

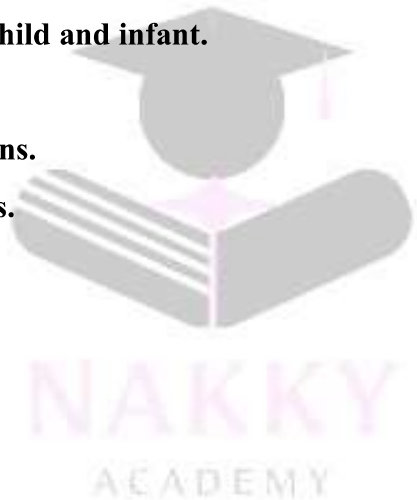
2023-2024



MODULE SIX: MEDICAL EMERGENCIES AND SAFETY.

After completing this module, you will be able to:

- **Monitor vital signs.**
- **Stop or minimize Hemorrhage.**
- **Demonstrate CPR Skills.**
- **Handle drowning.**
- **Save patients from heart attacks.**
- **Attend to cuts and wounds.**
- **Falls and Fire Safety.**
- **Handle Fit and/or epileptic seizure (Convulsions)**
- **Save a choking adult, child and infant.**
- **Handle spinal injuries.**
- **Attend to fires and burns.**
- **Manage asthma attacks.**



MEDICAL EMERGENCIES



Seniors or people with physical disabilities are more likely to experience some kind of accident or medical emergency. That's why it's important for seniors and their caregivers to have a plan in place to deal with an emergency when it happens. Let's review some steps you can take to prepare for a medical emergency and to ensure that you react calmly and purposefully when a crisis occurs.

Older adults make up an increasing share of emergency department (ED) users, as the proportion of those aged 65 years and older increases in the general population. Older adults often have physical, physiological, psychological, and social considerations which demand a different approach to evaluation, management, and discharge planning compared to most younger adults using the ED. Older people are also more likely to have accumulated morbidity throughout their life course.

Preparing the elderly for medical emergencies involves a combination of proactive measures and ongoing communication. Here are some steps that can help:

1. Create an emergency plan: Work with the elderly person and their family or caregivers to create an emergency plan that outlines what to do in case of a medical emergency. The plan should include important contact information, such as phone numbers for family members, doctors, and emergency services.

2. Ensure access to medical information: Keep a record of the elderly person's medical history, medications, allergies, and any other relevant information. This information should be easily accessible in case of an emergency.
3. Make sure they have emergency supplies: Keep a well-stocked first aid kit and any necessary medical supplies, such as oxygen tanks or medications, on hand.
4. Encourage regular check-ups: Regular check-ups can help identify health problems before they become emergencies. Encourage the elderly person to schedule regular check-ups with their doctor and follow their prescribed treatment plan.
5. Stay in regular communication: Regularly check in with the elderly person to see how they're feeling and if they have any concerns. This can help identify potential health problems early on and prevent emergencies.

Creating an emergency response plan for old people is an important part of preparing for potential emergencies. Here are some steps you can take to create an emergency response plan for an elderly person:

1. Identify potential emergencies: Identify the types of emergencies that are most likely to occur in the area where the elderly person lives. Common emergencies include medical emergencies, natural disasters, and fires.
2. Establish emergency contacts: Identify emergency contacts such as family members, neighbors, friends, or caregivers who can be contacted in case of an emergency. Make sure that each contact is aware of the elderly person's emergency response plan and their role in it.
3. Develop a communication plan: Identify a communication plan for emergencies, including how to contact emergency services and how to communicate with family members, caregivers, and other emergency contacts. Make sure the elderly person has a working phone and knows how to use it.

4. Create an emergency kit: Create an emergency kit that includes basic necessities such as food, water, medications, and first aid supplies. Make sure that the elderly person knows where the emergency kit is located and how to use its contents.
5. Establish evacuation routes: Identify evacuation routes for different types of emergencies and make sure the elderly person is familiar with them. Practice the evacuation routes to ensure that the elderly person can navigate them safely.
6. Review and update the plan regularly: Review and update the emergency response plan regularly to ensure that it remains relevant and effective. Make sure that all emergency contacts are up to date and that the elderly person's medical information and medications are current.

MONITORING VITAL SIGNS



When treating a casualty, you may need to assess and monitor his breathing, pulse, and level of response. This information can help you identify problems and indicate changes in a casualty's condition. Monitoring should be repeated regularly, and your findings recorded and handed over to the medical assistance taking over. In addition, if a casualty has a condition that affects his body temperature, such as fever, heatstroke, or hypothermia, you will also need to monitor his temperature.

Level of Response

You need to monitor a casualty's level of response to assess her level of consciousness and any change in her condition. Any injury or illness that affects the brain may affect consciousness, and any deterioration is potentially serious. Assess the level of response using the **AVPU scale** (below) and make a note of any deterioration or improvement.

A—Is the casualty Alert? Are her eyes open and does she respond to questions?

V—Does the casualty respond to Voice? Can she answer questions and obey commands?

P—Does the casualty respond to Pain? Does she open her eyes or move if pinched?

U—Is the casualty Unresponsive to any stimulus (i.e. unconscious)?

Breathing

When assessing a casualty's breathing, check the breathing rate and listen for any breathing difficulties or unusual noises. An adult's normal breathing rate is 12–16 breaths per minute; in babies and young children, it is 20–30 breaths per minute. When checking breathing, listen for breaths and watch the casualty's chest movements. For a baby or young child, it might be easier to place your hand on the chest and feel for movement of breathing. Record the following information:

Rate—count the number of breaths per minute.

Depth—are the breaths deep or shallow

Ease—are the breaths easy, difficult or painful?

Noise—is the breathing quiet or noisy, and if noisy, what are the types of noise?

PULSE

Each heartbeat creates a wave of pressure as blood is pumped along the arteries. Where arteries lie close to the skin surface, such as on the inside of the wrist and at the neck, this pressure wave

can be felt as a pulse. The normal pulse rate in adults is 60–100 beats per minute. The rate is faster in children and may be slower in very fit adults. An abnormally fast or slow pulse may be a sign of illness. The pulse may be felt at the wrist (radial pulse), or if this is not possible, the neck (carotid pulse). In babies, the pulse in the upper arm (brachial pulse) is easier to find. When checking a pulse, use your fingers (not your thumb) and press lightly against the skin. Record the following points;

Rate- number of beats per minute.

Strength- strong or weak.

Rhythm- regular or irregular.

Body Temperature

Although not a vital sign, low or high body temperature may be an important indicator of a life-threatening problem. You can feel exposed skin but use a thermometer to obtain an accurate reading. *Normal body temperature is 98.6°F (37°C) but can be slightly higher or lower. A temperature above 100.4°F (38°C) is usually caused by infection*, but can be the result of heat exhaustion or heatstroke. A lower body temperature may result from exposure to cold and/or wet conditions—hypothermia, or it may be a sign of life-threatening infection or shock.

There are several different types of thermometer; see below;

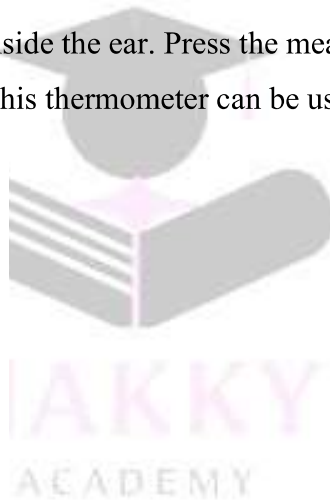
1. **Digital thermometer-** Used to measure temperature under the tongue or in the armpit. Leave it in place until it makes a beeping sound (about 30 seconds), then read the display



2. **Forehead thermometer-** A heat-sensitive strip for use on a small child. Hold against the child's forehead for about 30 seconds. The color on the strip indicates temperature.



3. **Ear sensor-** Place the probe inside the ear. Press the measurement key wait for a beeping sound, then read the display. This thermometer can be used while a person is asleep.



CONDUCTING CPR- UNCONSCIOUS ADULT

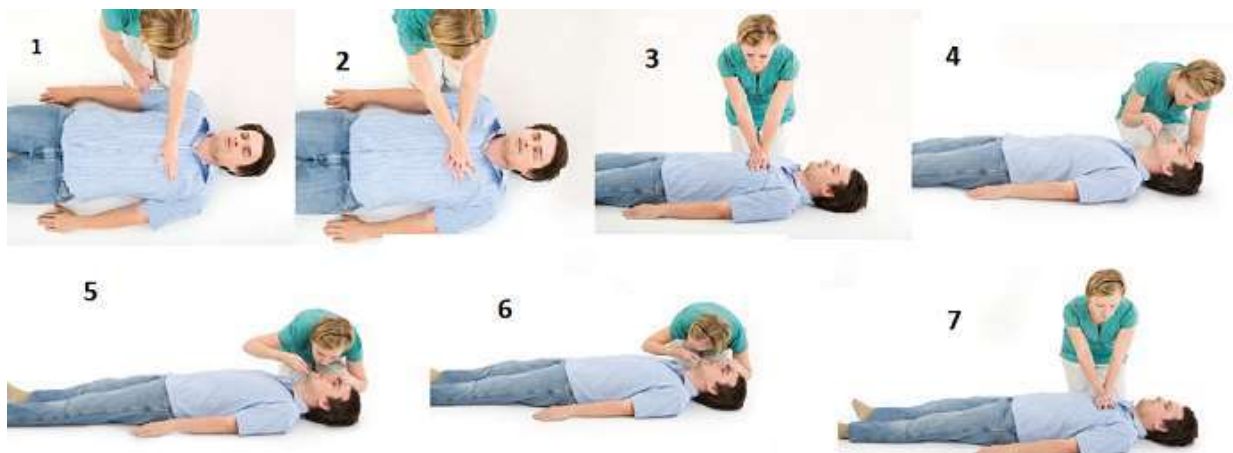
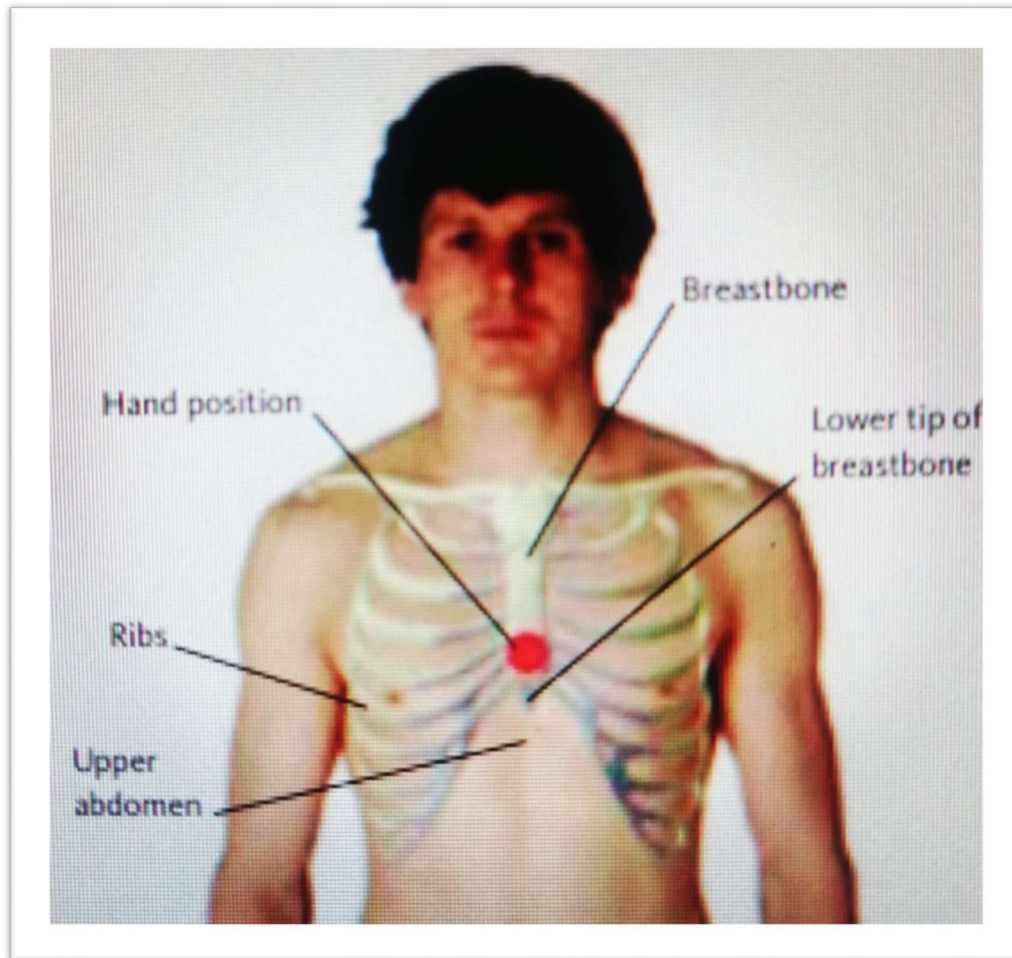


PHOTO CREDIT: RUTH JENKINSON, LIZZIE ORME

How to give CPR (cardiopulmonary resuscitation)

1. Kneel beside the casualty level with his chest. Place the heel of one hand on the center of the casualty's chest (Hand Position). You can identify the correct hand position for chest compressions through a casualty's clothing.
2. Place the heel of your other hand on top of the first hand, and interlock your fingers, making sure the fingers are kept off the ribs.
3. Leaning over the casualty, with your arms straight, press down vertically on the breastbone and depress the chest by at least 2 in (5 cm). Release the pressure without removing your hands from his chest. Allow the chest to come back up fully (recoil) before giving the next compression.
4. Compress the chest 30 times at a rate of at least 100 compressions per minute. The time taken for compression and release should be about the same.
5. Open the airway with the head tilt–chin lift maneuver: put one hand on his forehead and two fingers of the other hand under the bony tip of his chin. Move the hand that was on the forehead down to pinch the soft part of the nose with the finger and thumb. Allow the casualty's mouth to fall open.
6. Take a breath and place your lips around the casualty's mouth, making sure you have a good seal. Blow into the casualty's mouth until his chest rises. A complete rescue breath should take one second. If the chest does not rise, you may need to adjust the head position.
7. Maintaining head tilt and chin lift, take your mouth off the casualty's mouth and look to see his chest fall. If his chest rises visibly as you blow and falls fully when you lift your mouth away, you have given a rescue breath. Give a second rescue breath.
8. Continue the cycle of 30 chest compressions followed by 2 rescue breaths until: emergency help arrives and takes over; the casualty shows signs of regaining consciousness, such as coughing, opening his eyes, moving purposefully, and starts to breathe normally; or you are too exhausted to continue.

Special considerations for CPR

There are circumstances that make the normal steps for CPR more challenging.

For example:

- If you have not been trained in CPR or are unwilling or unable to give rescue breaths, you can give chest compressions only. If you call emergency services, the dispatcher will give you instructions for compression-only CPR.
- If there is more than one rescuer, change over every 2 minutes, with minimal interruption to chest compressions.
- If the casualty vomits during CPR, roll him away from you onto his side, ensuring that his head is turned toward the floor to allow vomit to drain away. Clear any residual debris from his mouth, then immediately roll him onto his back again and recommence CPR.
- If a woman in the late stages of pregnancy requires CPR, raise her right hip off the ground by tilting it upward before you begin chest compressions (see below).
- Modified rescue breathing may be necessary in some cases: for example, if a casualty has mouth or oral injuries, you can give rescue breaths through the nose (opposite). A casualty may breathe through a hole in the front of the neck—a stoma (opposite). You can also use a pocket mask or face shield when giving rescue breaths

UNCONSCIOUS CHILD

How to give CPR

1. Determine unresponsiveness and assess breathing (not just gasping).
2. Kneel level with the child's chest. Place one hand on the center of her chest. This is the point at which you will apply pressure.
3. Lean over the child, with your arm straight, and then press down vertically on the breastbone with the heel of your hand. Depress the chest by at least one-third of its depth—about 2 in (5 cm). Release the pressure without removing your hand. Allow the chest to come back up completely (recoil) before you give the next compression. Compress the chest 30 times, at a rate of at least 100 compressions per minute. The time taken for compression and release should be about the same.

4. Open and assess the airway. Pick out any visible obstructions from the mouth. Do not sweep the mouth with your finger to look for obstructions.
5. Pinch the soft part of the child's nose with the finger and thumb of the hand that was on the forehead. Make sure that her nostrils are closed to prevent air from escaping. Allow her mouth to fall open.
6. Take a deep breath in before placing your lips around the child's mouth, making sure that you form an airtight seal. Blow steadily into the child's mouth for one second; the chest should rise.
7. Maintaining head tilt and chin lift, take your mouth off the child's mouth and look to see the chest fall. If the chest rises visibly as you blow and falls fully when you lift your mouth, you have given a rescue breath. If the chest does not rise you may need to adjust the head. Give TWO rescue breaths.
8. If you are alone, perform chest-compression only CPR or CPR using cycles of 30 compressions followed by TWO rescue breaths, for two minutes, then stop to call 911 for emergency help. Continue CPR until emergency help takes over, the child starts to breathe normally, or you become too exhausted to continue.

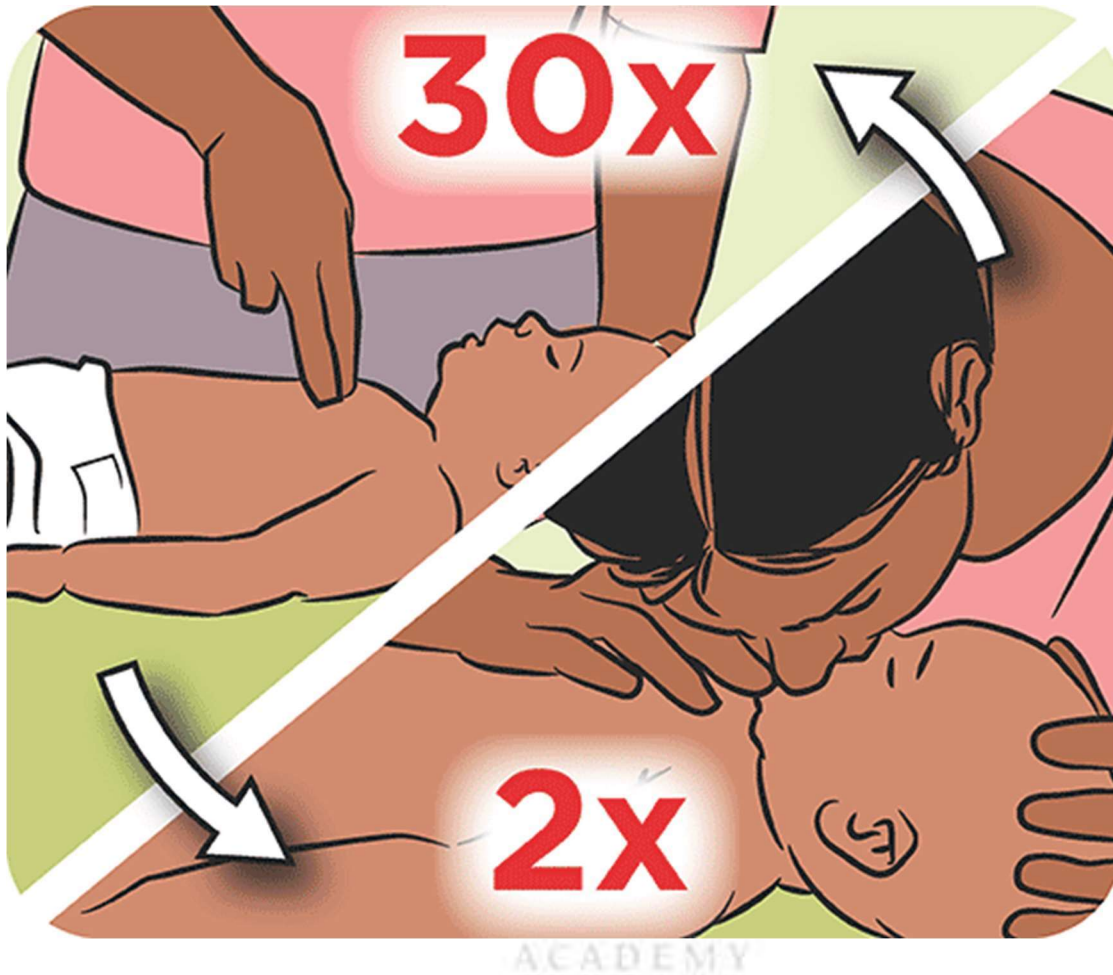
UNCONSCIOUS INFANT

First check responsiveness and assess for breathing; if absent, give compressions immediately. Gently tap or flick the sole of the infant's foot and call his name to see if he responds. ***Never shake an infant.***

How to give CPR.

1. Place the infant on his back on a flat surface, at waist height in front of you or on the floor, with a rolled-up towel under his shoulders.
2. Place two fingertips of your lower hand on the center of the infant's chest. Press down vertically on the infant's breastbone and depress the chest by at least one-third of its depth—about 1½ in (4 cm). Release the pressure without losing the contact between your

fingers and the breastbone. Repeat to give 30 compressions at a rate of at least 100 times per minute.



3. Open the airway by putting one hand on the infant's forehead and a fingertip of the other hand under the tip of his chin and tilting up and back as shown.
4. Pick out any visible obstructions from mouth and nose. Do not sweep the mouth with your finger looking for obstructions; this could push an object farther down.
5. Take a breath. Place your lips around the infant's mouth and nose to form an airtight seal. If you cannot make a seal around the mouth and nose, close the infant's mouth and make a seal around the nose only. Blow steadily into the infant's mouth for one second; the chest should rise. Release your mouth and watch to see that the chest falls. Give a second breath.
6. If you are on your own, continue alternating 30 chest compressions with TWO rescue breaths for two minutes, then stop to call 911 for emergency help. If help is on the way,

continue CPR until emergency help arrives and takes over, the infant starts to breathe normally, or you become too exhausted to continue.

If normal breathing resumes at any stage, hold the infant in the recovery position (below).

